

Stress Urinary Incontinence (SUI)

WARWIKI

Patient Instructions · Leaking with coughing, laughing, lifting, or exercise

Stress urinary incontinence, or SUI, is leaking urine when something puts sudden pressure on the bladder — coughing, sneezing, laughing, lifting, or exercise. It happens when the support under the urethra weakens. It is very common and very treatable, with options ranging from simple exercises to a quick surgery.

About This Condition

The **urethra** (the tube urine flows through) normally stays closed when you cough or strain, thanks to the support of the pelvic floor. When that support weakens, a sudden rise in belly pressure pushes urine out. The hallmark is **leaking with activity** — not with a sudden urge (that is overactive bladder; some people have both, called **mixed** incontinence).

What Causes It

Anything that weakens or stretches the pelvic-floor support:

- **Pregnancy and childbirth**
- **Menopause** and aging
- Prior pelvic surgery (in men, often after prostate surgery)
- Chronic coughing, heavy lifting, or extra body weight

LEARN THE TERMS

Stress incontinence

Leaking urine with pressure — coughing, lifting, exercise.

Urethra

The tube that carries urine out of the body.

Pelvic floor

The muscles that support the bladder and urethra.

Kegels

Pelvic-floor squeezing exercises that build support.

Mixed incontinence

Having both stress leaks and urge leaks.

Pessary / insert

A small support placed in the vagina to reduce leaks.

Urethral bulking

An office injection that helps the urethra seal.

Sling

A small support placed under the urethra in surgery.

DO I NEED SURGERY? Not necessarily. Many people improve with pelvic-floor exercises and simple changes, and some choose a support device. Surgery (like a sling) is very effective when you want a longer-lasting fix — but it is your choice, and you can start with the simpler steps.

How It's Diagnosed

- A talk about when you leak, and an exam (sometimes a cough test)
- A **urine test** to rule out infection
- A **bladder diary**, and a check of how well you empty
- Sometimes bladder-function testing (urodynamics) before surgery

How It's Treated (Step by Step)

- 1 **Pelvic-floor exercises** (Kegels) and physical therapy, weight loss, and treating a cough or constipation.
- 2 **Support devices** — a vaginal pessary or insert to use as needed.
- 3 **Urethral bulking** — a quick office injection that helps the urethra seal.
- 4 **Surgery** — a **mid-urethral sling** or other procedure for a lasting repair. Each option has its own handout.

Living With SUI

- Learn to **squeeze before you cough, sneeze, or lift** ("the knack").
- Do pelvic-floor exercises daily — it can take 6–12 weeks to see gains.
- Treat constipation and a chronic cough, which add pressure.

Call your team if you have:

- Burning, urgency, or **blood in the urine** (possible infection)
- Leaking with a **strong urge** too — treatment may differ
- Trouble emptying, or a sense the bladder isn't emptying

THREE THINGS TO REMEMBER

1. SUI is leaking with pressure (cough, laugh, lift, exercise) from weakened support under the urethra — common and very treatable.
2. Options go from pelvic-floor exercises and a support device, to an office bulking injection, to a sling for a lasting fix.
3. Pelvic-floor exercises take weeks to work; learn to squeeze before you cough or lift. Surgery is a choice, not a must.